

Disaster Psychiatry and Post Traumatic Stress Disorder

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Do.
Make.
Heal.
Innovate.
Reinvent the Future.

Session Objectives

1. Students will be able to describe possible acute response to trauma
2. Students will be able identify groups at risk for a traumatic stress reaction.
3. Students will be able to appreciate initial and psychological First Aid interventions to a disaster situation.
4. Students will be able to describe the main symptoms clusters of PTSD.
5. Students will know the importance of screening for PTSD and what the options are for the initial management of patients with PTSD.

Initial Mental Health Intervention

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- Protect
- Direct
- Connect

What is a Disaster?

“A severe disruption, ecological and psychosocial which greatly exceeds the coping capacity of the affected community.”

Mental Health Issues Coronavirus

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- Social Distancing ➡ boredom, loneliness, isolation, depression
- Quarantine ➡ insomnia, reduced feelings of safety, scapegoating, Alcohol/Tobacco
- Media ➡ amplifies Social Media-can misinform or be helpful
- Fear and uncertainty of the unknown
- Anxiety and guilt and stigma
- Protecting mental health of healthcare workers
- Telemental health, smart phone
- Anticipate and address and counsel about stress reaction
- Long COVID/Neuropsychiatric Sequelae

Source: Jeffrey Goldberg, DO

Actions You can Take for your wellbeing during Coronavirus Pandemic

- Keep a structure, schedule even while self-isolated
- 30 min of aerobic exercise
- Practice Deep Breathing and relaxation techniques
- Limit media exposure, news smart phone alerts
- Do use phone/Internet to video chat and connect with others
- We quell anxiety with bonding and connecting with others

Types of Disasters: Human Caused

- Plane crash
- Terrorism incidents (e.g. Bombings)
- Nuclear accidents
- Shootings
- Riots
- Arson
- Immigration emergencies
- Environmental
- Chemical/
biological

Types of Disasters: Natural

- Earthquake
- Tsunami
- Landslide
- Famine
- Floods
- Volcanic Eruptions
- Tropical Storms
- Wildfires

- Natural Disasters



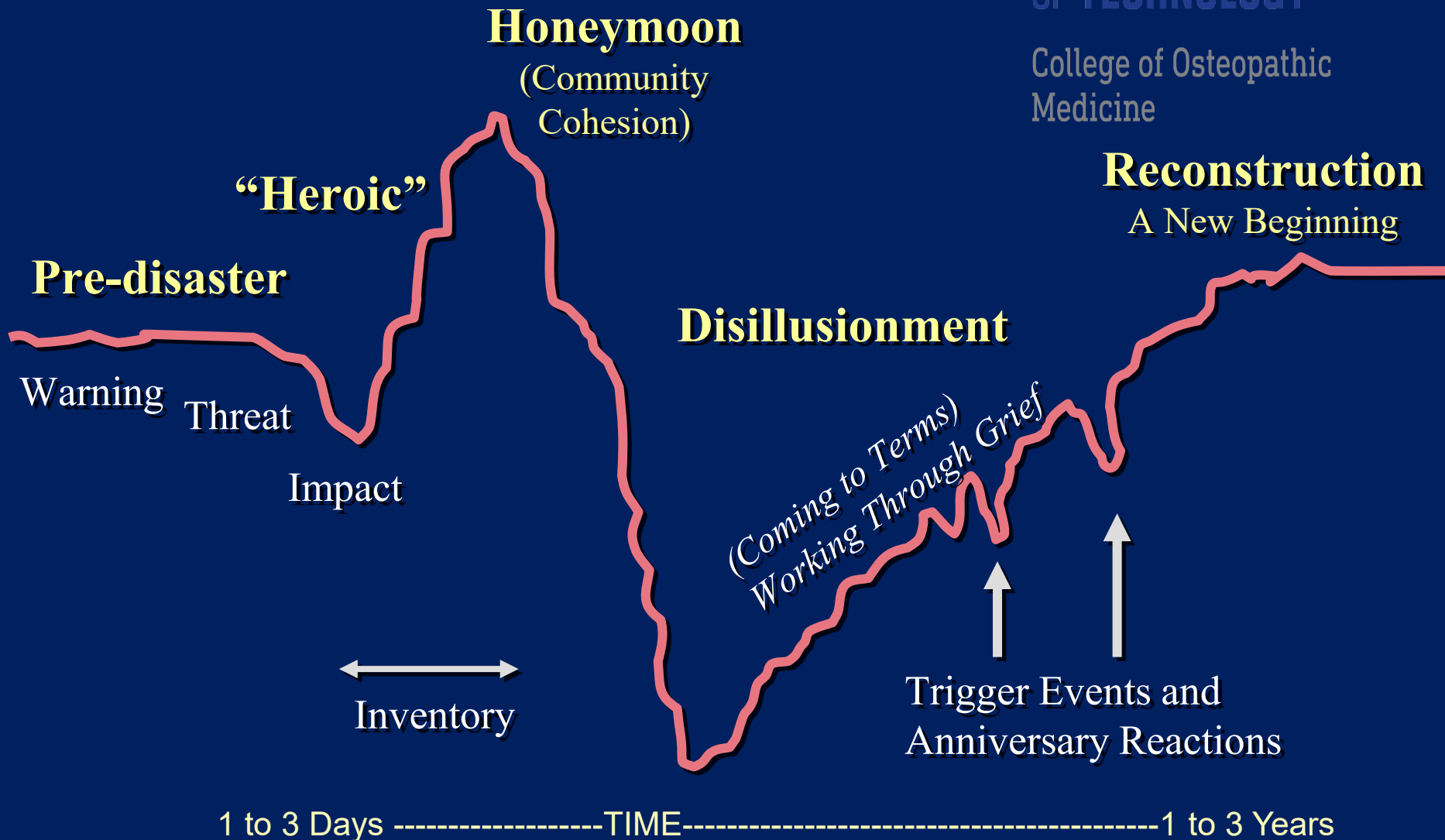
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Disaster Psychiatry

An epidemiological
approach to understanding
and treating the effects of
mass/casualty situations.



• Source: Zunin/Meyers

Disruption of Medical Services

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1. Unable to get medicine
2. Unable to Use Respiration Equipment
3. Roads Damaged
4. Facility Closed
5. Doctors Unavailable
6. Unable to Afford Care
7. Transport Issues
8. No 911 Service
9. Unable to Have Dialysis

<https://www.nejm.org/doi/full/10.1056/NEJMsa1803972>

- **Key Concepts of Disaster Impact**

No one who sees a disaster is untouched by it.

- Primary, Secondary and Tertiary victims

Two types of trauma

- Individual (*stress and grief reactions*)
- Collective (*Damages the bonds and social fabric of the community. Increases fatigue, irritability, family conflict.*)

• Key Concepts of Disaster Impact

- People pull together during and after a disaster. (*high activity/low efficiency*)
- Stress and grief are normal reactions to an abnormal situation. (*transitory reactions*)
- Emotional reactions relate to problems of living. (*abnormal and excessive disruptions to daily routines*)

Key Concepts of Disaster Impact

- Disaster relief – can seem complex and overwhelming
- People typically do not seek out mental health services (*self reliance at all costs*)
- Survivors reject help (*others need it more than I do*)
- Mental health services are practical rather than psychological

• Critical Disaster Stressors

- Threat to one's life
- Threat of harm to family
- Destruction of home or community
- Significant media attention
- Witnessing others' trauma
- Being trapped or unable to evacuate
- Living with uncertainty

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Source: Adapted from CMHS

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Factors Which Influence Recovery

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- Natural vs Human caused
- Degree of personal impact
- Size and scope of severity of event
- Probability of recurrence
- Community disruption/destruction combined with personal loss

Key Events with Mental Health Implications

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- Death Notification
- Ending search and recovery
- Criminal justice proceedings
- Returning to impacted areas
- Funerals and memorials

Reactions to a Disaster

- Cognitive
- Emotional
- Behavioral
- Physical

Cognitive Reactions to a Disaster

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- Trouble concentrating or remembering things
- Difficulty making decisions
- Preoccupation with the event
- Recurring dreams or nightmares
- Questioning spiritual beliefs

• Emotional Reactions to a Disaster

- Feeling depressed or sad
- Feeling irritable, angry, resentful
- Experiencing anxiety or fear
- Feeling despair or hopelessness
- Being apathetic
- Feeling overwhelmed

• Behavioral Reactions to a Disaster

Isolation from others

Sleep problems

Increased conflicts with family

Hyper-vigilance, startle reactions

Avoiding reminders

Crying easily

Not eating

- **Physical Reactions to a Disaster**

Headaches

Generalized discomfort, hot or cold

Hypertension, heart pounding

Gastrointestinal distress

Exacerbation of psychiatric illness

Accelerated physical decline

Fatigue or exhaustion

Increase or decrease in appetite

- **Mental Disorders Secondary to Medical Condition**

- Head Injury
- Toxic Exposure
- Illness / Infection
- Dehydration
- Loss of Medications
- Withdrawal Symptoms

Comparison of Physical/Emotional Symptoms in Months Following 9/11

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<u>Symptoms</u>	NTS 2001		NTS 2002	
	National	New York	National	New York
Didn't feel like eating	28.8	46.4	15.0	16.2
Smoked more than usual	21.2	20.1	10.5	13.8
Had headaches	21.7	24.3	17.3	25.1
Had upset stomach	36.7	34.8	19.0	16.1
Cried	60.3	74.1	20.6	31.6
Had trouble getting to sleep	51.2	59.4	30.9	39.0
Felt very nervous or tense	49.9	62.5	26.9	33.6
Felt like getting drunk	6.6	11.9	8.6	10.7
Felt more tired than usual	37.5	47.6	32.5	41.2
Felt dizzy at times	9.0	13.3	10.9	19.8
Lost my temper more than usual	19.9	27.2	17.7	20.3
Hands sweated and felt damp and clammy	9.4	10.6	6.0	10.2
Had rapid heart beats	16.0	21.6	11.3	20.5
Felt sort of dazed and numb	45.7	46.3	12.3	17.3
Kept forgetting things	19.7	27.9	21.1	34.0
None of these	9.6	2.9	37.9	22.6

Source: University of Chicago, National Opinion Research Center, National Tragedy Survey 2001-2002

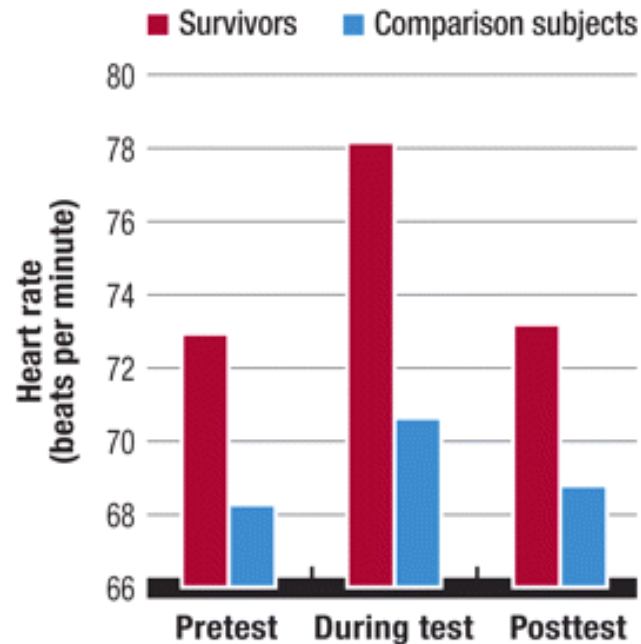
- **Oklahoma City Bombing:
Reactions Still Felt Years Later...**

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**Blast Still Felt
In the Heart**

Seven years after the Oklahoma City federal building was bombed, survivors still had higher heart rates before, during, and after reminders of the event than unaffected control subjects.



Source: Phebe Tucker, M.D., et al., *American Journal of Psychiatry*, February 2007

- **Key Concepts to Remember**

The target population is normal

Avoid mental health labels

Be innovative in offering help

Fit program services into the community context

• Special Populations

Children and youth

Older adults

People with disabilities

People with serious mental illness

People with low socioeconomic status

Cultural and ethnic groups

Disaster workers

- **Factors Affecting Children's Recovery From a Disaster**

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Developmental level of child

Pre-disaster mental health of the child

Ability of the community to offer support

Separation from parents

Reaction of significant adults to the disaster



- **Factors Affecting Children's Recovery From a Disaster**

Communication between
parents and child

Belief about what caused
the disaster

The degree of
damage/Violence cause
by the disaster

The degree to which the
child was directly
impacted by the disaster

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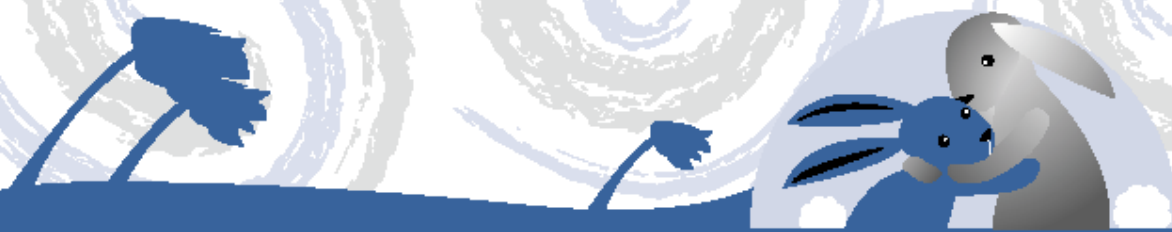
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Children's Reactions to a Disaster

- **School-Age Children**

- Physical complaints
- Fears about safety
- Nightmares
- Poor concentration
- Preoccupation with event
- Fighting
- Depression
- Guilt
- Withdrawal from peers



The most confusing thing was _____

The scariest thing was _____

The saddest thing I saw was _____

Something I heard grownups say was _____

Some people were doing really good things to help others.
One good thing that happened was _____

How I Want My Hurricane Story to End

Can you believe all the damage the hurricane has done? It can be so hard at times to understand and live through a hurricane like that. Yet, you will probably learn a lot about yourself and your family and how to deal with a hurricane disaster. After a while, you may begin to feel stronger and safer, and proud of things that you did to help yourself, your family, and your community.

Write or draw a picture of yourself now, and then imagine how you would like to be in the future. By then, you may have grown, learned a lot, and changed in many ways.

Now

In The Future



Older Adults Reactions to a Disaster: Impact of Losses

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- Poor health
- Need assistance in daily living
- Isolation
- Poor support system
- Limited income



The Dallas Morning News / Barbara Davidson via AP

Older Adults Reactions to a Disaster

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- Intense sense of grief over mementos, pets, etc.
- Feel unable to start over
- Past losses re-awakened
- Slower to respond to the impact of the loss
- Experience a long-term decline in standard of living



Older Adults Reactions to a Disaster: Stress Symptoms

- Slower to recover psychologically and financially
- Fear of loss of independence
- Depression
- Withdrawal
- Agitation
- Sleep disturbance
- Memory loss
- Disorientation and confusion
- Apathy

People with Mental Illness

- Same basic needs as everyone else
- May have special needs
- May “rise to the occasion”
- Program should tailor services to ensure appropriate services are delivered
- Often identify unserved or underserved people in completing outreach

People with Mental Illness

- Disaster stress reactions may be difficult to discern from symptoms of mental illness
- Consumers may be trained as part of the preparedness process
- Should be given the opportunity to serve the larger community

Disaster Workers

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- Emergency Medical Services
- Law Enforcement/Fire Service
- Emergency Management
- Voluntary Agencies
- Utility Workers



Mobile Register / Victor Calhoun

Disaster Workers

- Train peers when possible
- Recognize unique stressors
 - Conflicting roles (*family vs. job*)
 - Exposure to chaos, death and destruction
 - Very long days, exhaustion
 - Community reactions (*hero or scapegoat*)

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The Tampa Tribune / Michael Spooneybarger

Stress Management for Healthcare Providers

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- Monitoring basic needs
- Clear communication
- Regular breaks
- Participation in normal life events
- Contact with loved ones
- Place for support
- Respect differences
- Preparedness

• Psychological First Aid

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Helping Victims in the Immediate Aftermath of Disaster

Do's:

- Promote Safety
- Promote Calm
- Promote Connectedness
- Promote Self-Efficacy
- Promote Hope

• Psychological First Aid

Helping Victims in the Immediate Aftermath of Disaster

Don'ts:

- Force people to share their stories with you, especially very personal details.
- Give simple reassurances like “everything will be OK” or “at least you survived.”
- Tell people what you think they should be feeling, thinking or doing now or how they should have acted earlier.

• Psychological First Aid

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Helping Victims in the Immediate Aftermath of Disaster

Don'ts (continued):

- Tell people why you think they have suffered by alluding to personal behaviors or beliefs of victims
- Make promises that may not be kept
- Criticize existing services or relief activities in front of people in need of these services

• Pharmacotherapy

Sleep Induction

Reduction of Anxiety

Acute Stress Disorder Symptoms

- Antidepressants?
- Propranolol?
- Risperidone?

No evidence of a PTSD protective benefit with benzodiazepines

Summary Statements

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Rescue workers and mental health teams can play a vital role in ameliorating the mental health problems of disaster survivors through:

- Awareness of risk factors
- Appropriate screening
- Initial Mental Health Intervention
- Psychological First Aid

PTSD

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